



Neurological Issues affecting IP claims

the grey areas & blurred boundaries between neurology and psychology

Dr Brian Marien

Health Care Forum

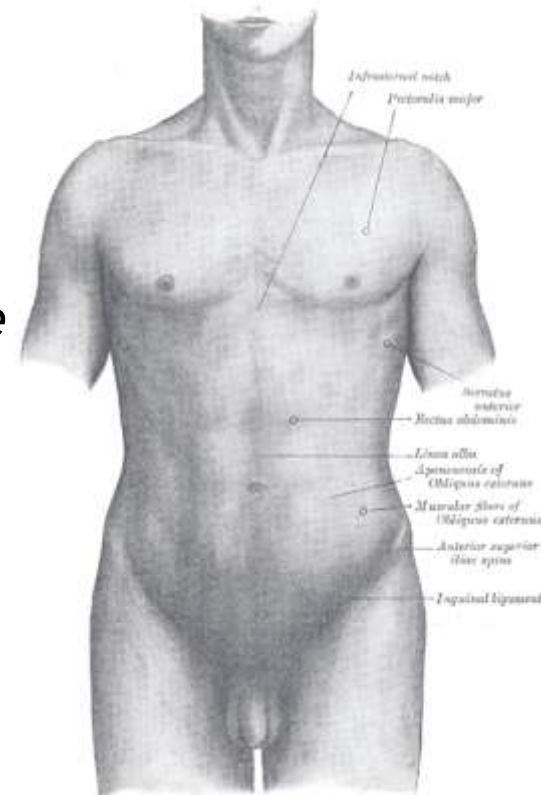




The Disease Model

Disease is biological event with pathology, a specific entity, concrete and identifiable.

'a morbid entity characterised usually by two of these criteria: recognised aetiological agent, identifiable group of symptoms or signs, or consistent anatomical alterations'.





‘medically unexplained physical symptoms’ (MUPS)

MUPS/Functional Somatic Syndromes are physical illnesses without an organic explanation and devoid of demonstrable structural lesion or established biochemical change. (Sharpe et al., 1995)



Medically unexplained physical symptoms (MUPS)



MUPS are common

MUS accounts for 30-70% of all consultations in primary care & 35-50% of all new medical outpatients (RCP)

MUPS are expensive

frequent consultations (in primary and secondary care)
polypharmacy
repeated investigations, hospital admissions
associated with high healthcare costs –
prolonged distress & disability.

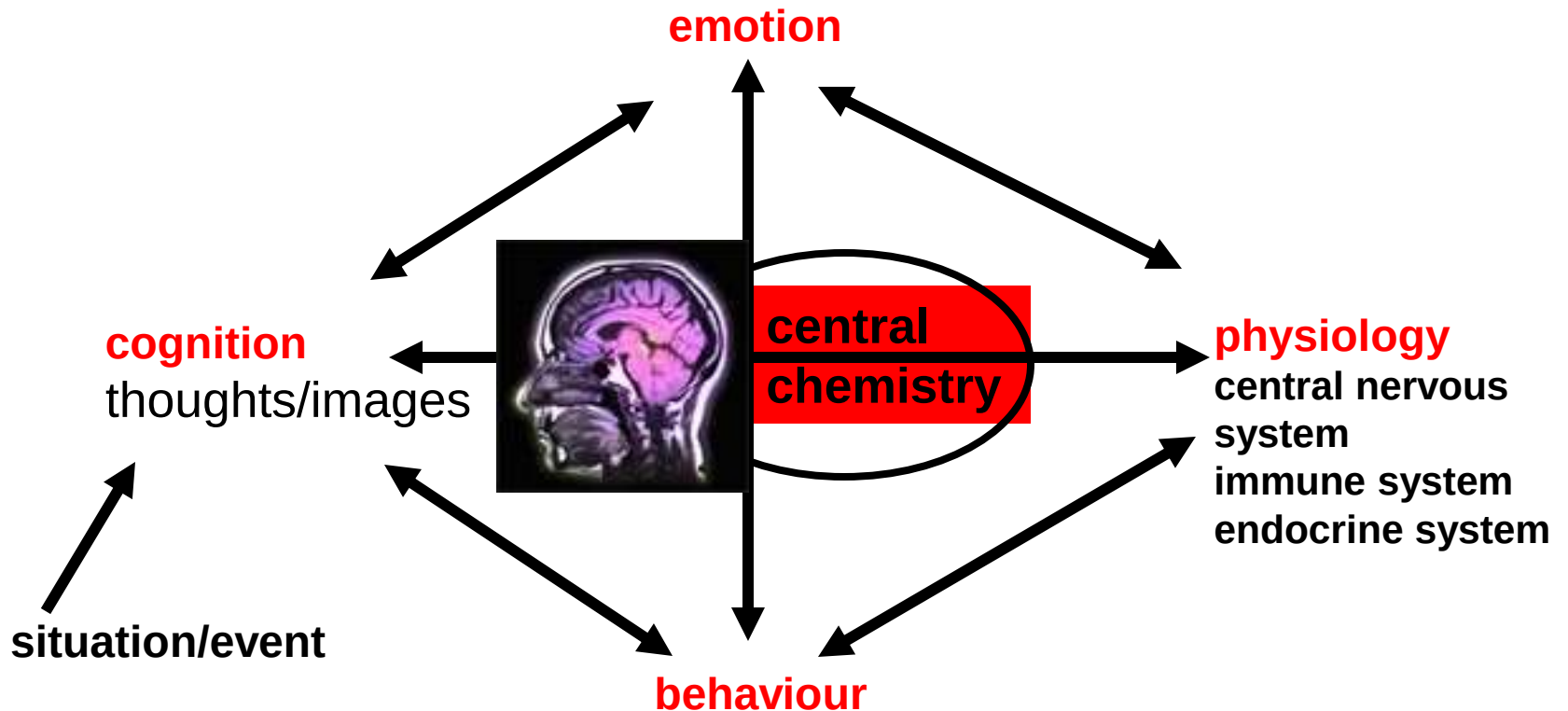




the ghost in the machine

The cognitive behavioural approach is an integrative model

linking our thinking, our brain and body chemistry, our feelings and our behaviour



Case Study

40 year old male, senior executive working long hours experiences episodes of dizziness.
Seen by a neurologist – extensive Ix = nad
Rx vestibular exercises; medication
2nd & 3rd opinions – more Ix = nad *‘revolving door’*

Worsening dizziness along with poor concentration, difficulty making decisions, poor memory, fatigue, headaches, un-refreshing sleep...



Outcome

Stops work – signed off by GP
Income protection @ 6 months

Offered referral to psychiatrist – refuses

Work history – extended period of work stress, problems with line manager, periods of work absence, anxiety, IBS

Have you ever felt stressed or anxious?



- ☐ How do you know you are stressed?
- ☐ How does it effect the way you feel? – physical symptoms
- ☐ How does it effect the way you think?
- ☐ How does it effect the way you behave?

physical symptoms commonly associated with stress/anxiety

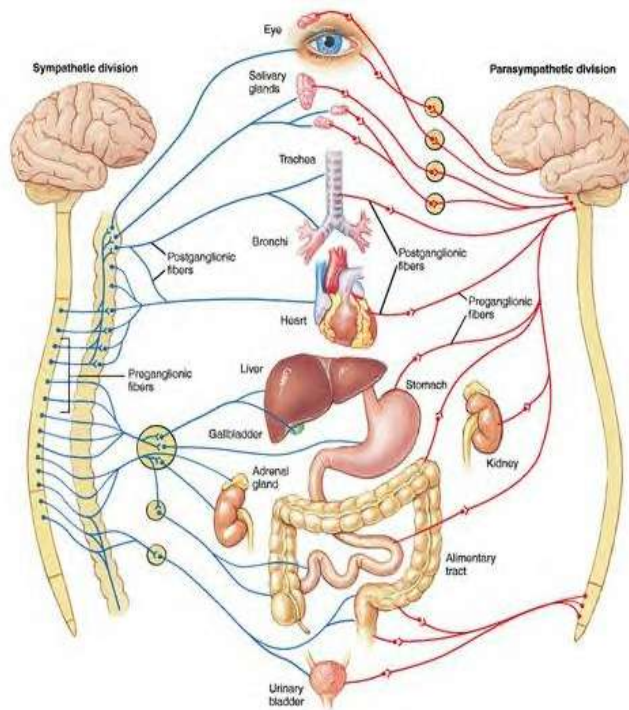


- chest tightness; throat tightness
- rapid heart rate; palpitations; sweating..
- muscle tension- causing localised discomfort = headache, neck stiffness,
- musculo-skeletal pain
- breathlessness (hyperventilation) – dizziness, light-headed, numbness, pins and needles, tingling, faintness, tremor, feeling strange or detached;
- indigestion; dyspepsia; nausea; heartburn; diarrhoea; irritable bowel syndrome; dry mouth;
- migraine;
- fatigue
- skin rashes.
- urinary frequency
- increased physical sensitivity to light, touch, pain & sound

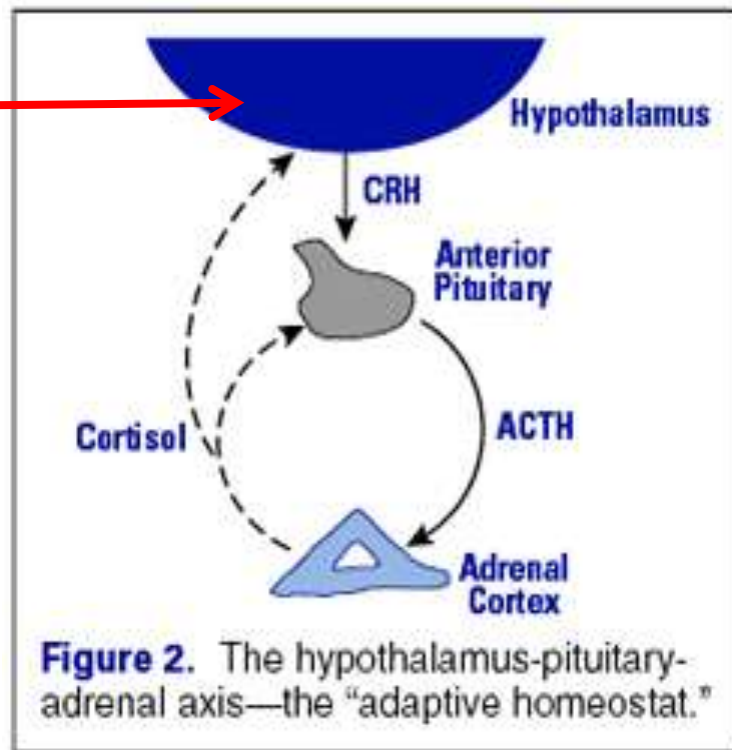
impacts on health, well-being, engagement, & performance



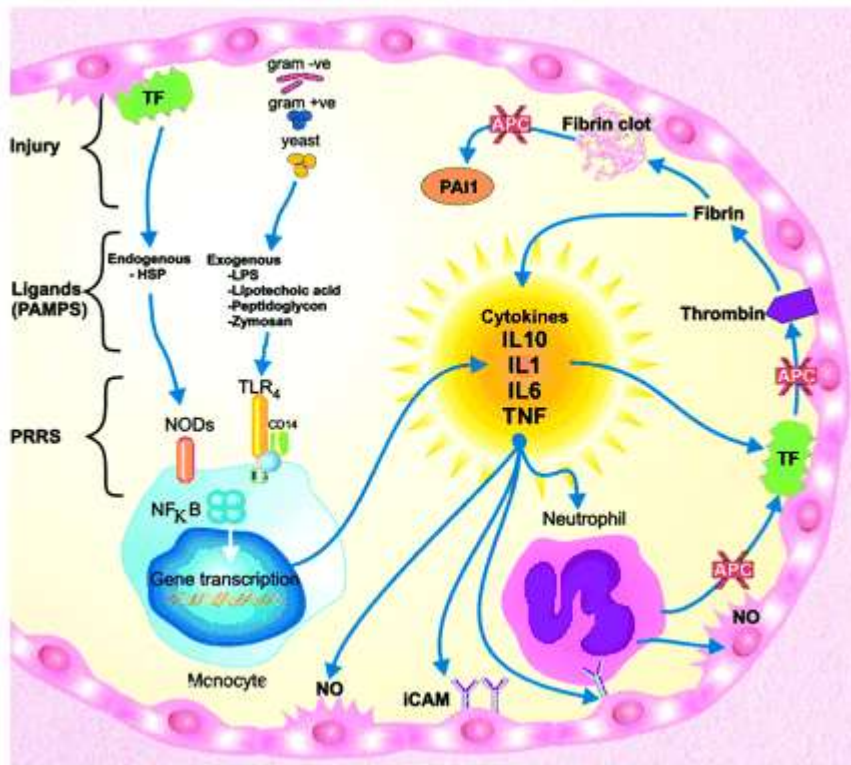
Emotions are embodied



Stress
Anxiety
Depression



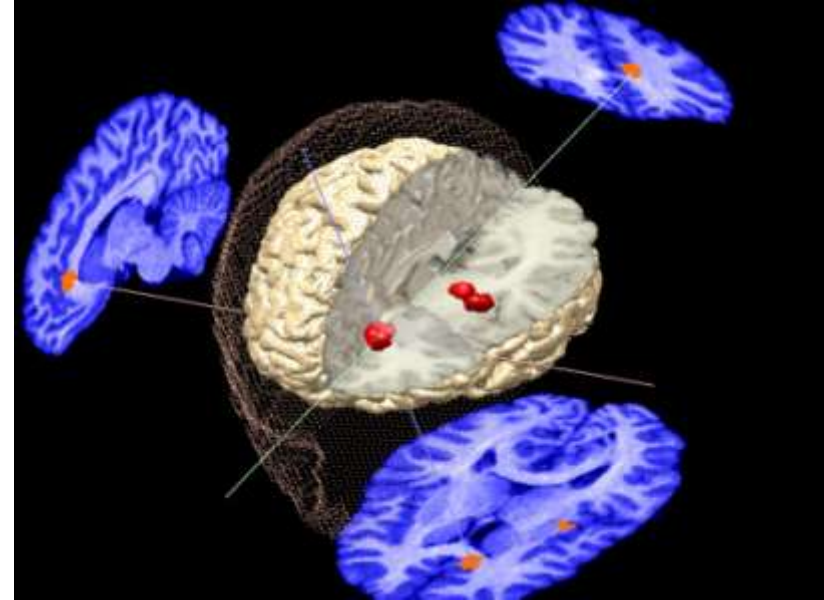
endocrinology - hormonal changes



stress & the immune system
cortisol; pro-inflammatory cytokines

cognitive impact of stress & worry

- sense of fear & apprehension;
- focus on worry/threat – attentional hijack
- difficulty concentrating; impaired judgement
- impaired memory – forgetfulness
- difficulty in making decisions
- negative thoughts re self and others
- distorted, irrational & ‘catastrophic’ thoughts – rumination



‘cognitive fitness’

impacts on engagement, cognitive fitness & performance

behavioural symptoms of worry, stress & anxiety

- inner tension, restlessness, unable to settle/unwind
- irritable, intolerant, critical, blaming
- avoidance of anxiety provoking situations
- low productivity, 'presenteeism'
- work absence
- sleep disturbance; loss of libido
- social withdrawal
- dysfunctional coping strategies:- alcohol, smoking, drug taking, eating etc



impacts on health, engagement, cognitive fitness & performance

Testing their patients Why are so many of us complaining about our doctors? By Denis Campbell

The patient-doctor relationship can be complicated. It needs honesty, confidence, and an ability to listen and communicate. Yet last week it emerged that written complaints about NHS hospital and community health services in England had risen more sharply than ever before. The total jumped from 89,139 to 101,077 – a rise of 13.4%. The largest single group of complaints (44% of all cases), involved doctors – double that of nurses, midwives and health visitors combined. The most common complaint was about treatment patients had received (42%), then the attitude of staff (12%).

So what's going wrong? "For patients and doctors to have a good relationship they need to trust each other, which comes down to personal rapport and openness," says Dr Paul Hodgkin, a part-time GP and chief executive of Patient Opinion, a website for NHS patients to record their experiences. "People hate doctors when they don't tell them the truth and feel that they haven't been treated as equals. Doctors are so much better [at this] than they used to be, but they could be a lot better."

Katherine Murphy, director of the



'This won't hurt a bit'... Patients and doctors need to trust each other

"The patient can be upset, angry and annoyed with you because by saying it's not a physical problem you aren't fulfilling their needs," says Field. Some accept psychological treatment, but others move to another GP.

By far the most frightening reason for a relationship breakdown

discs in the neck, which turned into a spinal abscess and then pressed on her spinal column, which is serious," recalls Luscombe. "This condition was missed by her GP for about four weeks, and if it had not been spotted for a few days more, she would potentially have ended up paralysed, though she still

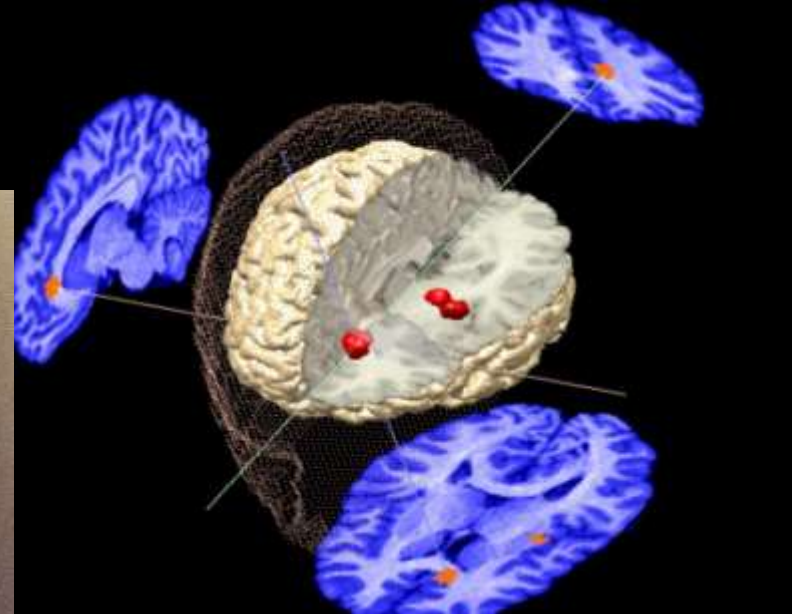
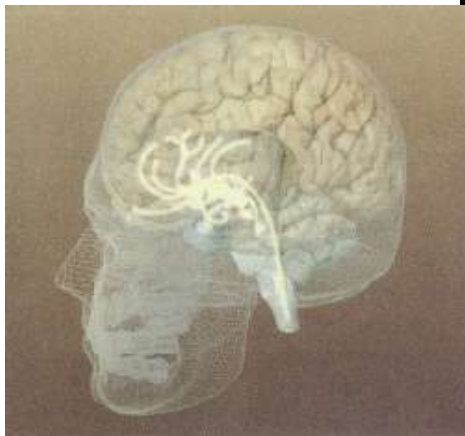
Emotions are Hard-wired

stress/anxiety/fear

think

feel

behave



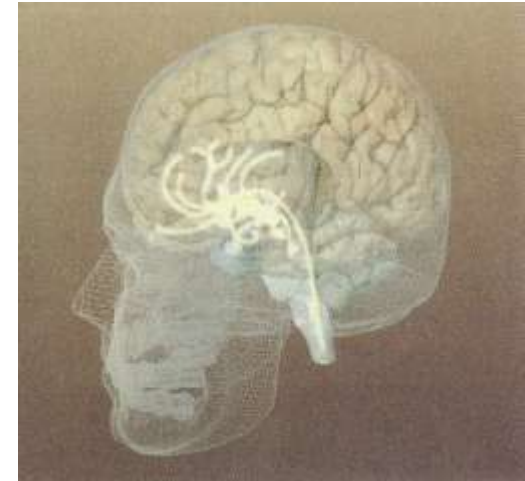


sad / low/ depressed

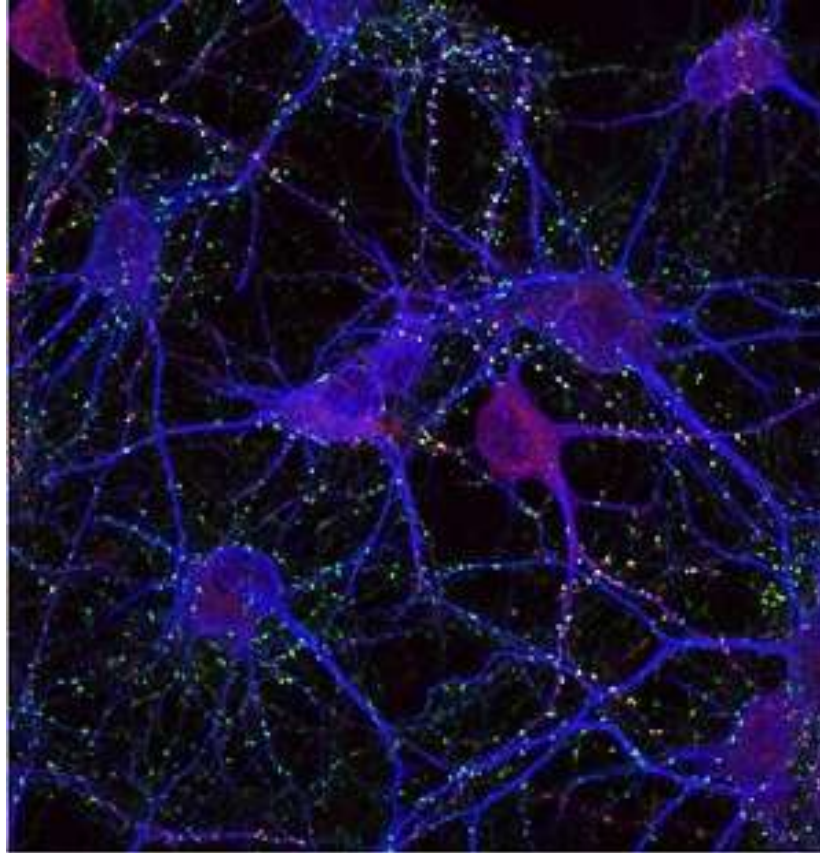
think

feel

behave

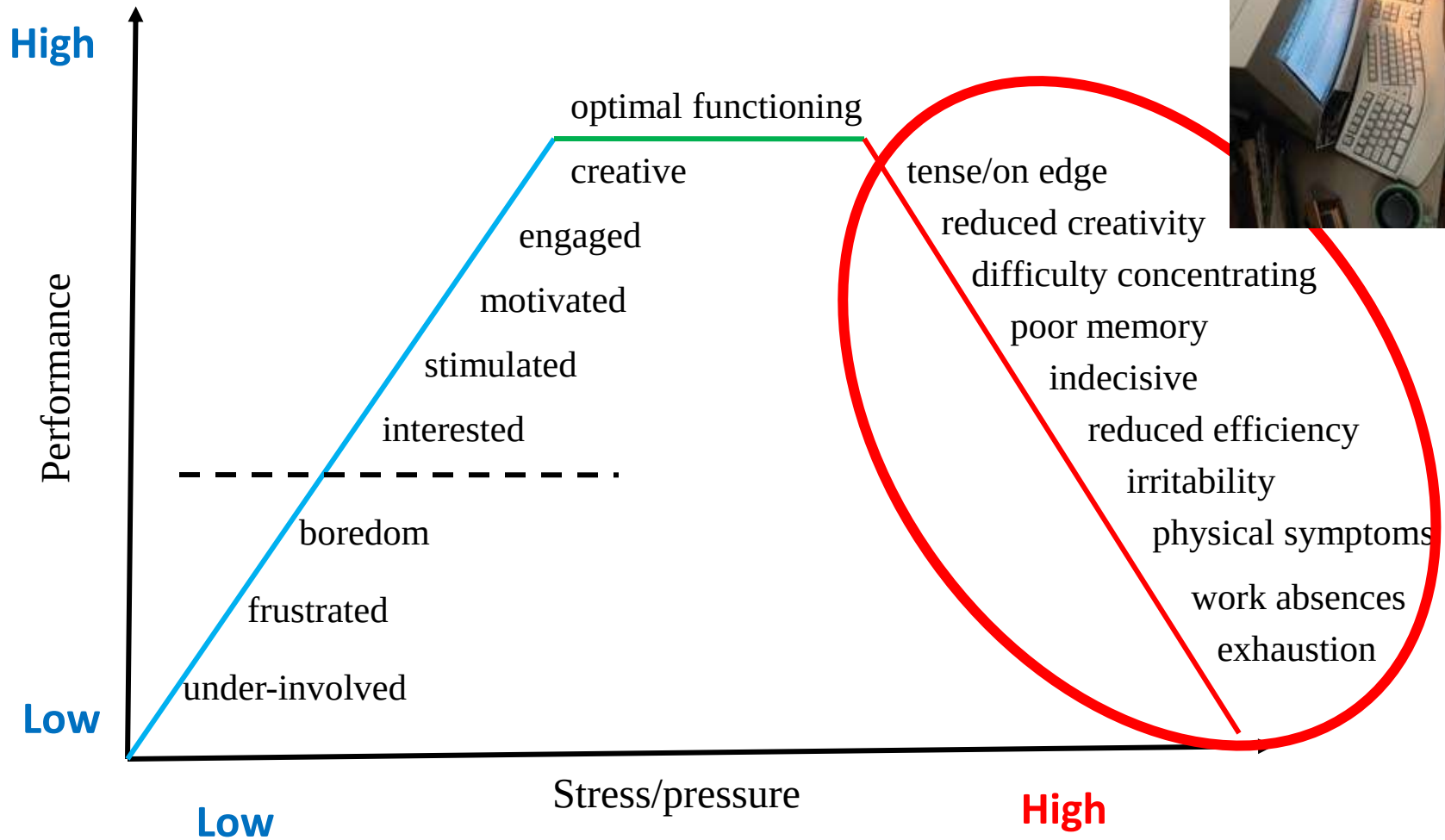


neurons that fire together wire together



neuroplasticity

psychological well-being & performance



Case Study

40 year old male, senior executive working long hours experiences episodes of dizziness.
Seen by a neurologist – extensive Ix = nad
Rx vestibular exercises; medication
2nd & 3rd opinions – more Ix = nad

Worsening dizziness along with poor concentration, difficulty making decisions, poor memory, fatigue, headaches, un-refreshing sleep...



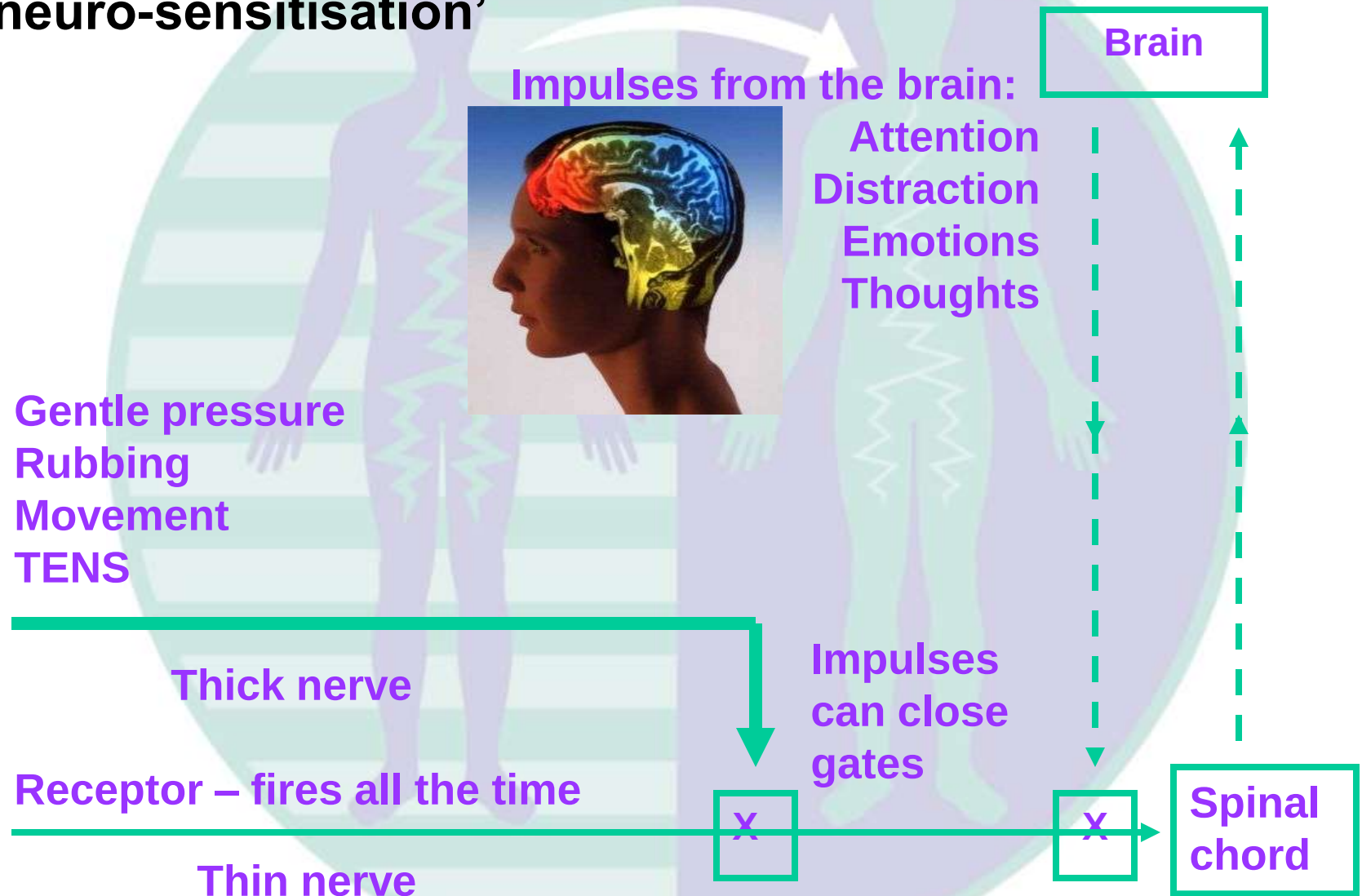
Stops work – signed off by GP
Income protection @ 6 months

Offered referral to psychiatrist – refuses

Work history – extended period of work stress, problems with line manager, periods of work absence, anxiety, IBS

Gate Control Theory of Pain

‘neuro-sensitisation’



Emotional memories

*events that are associated with emotion
are encoded into long-term memory (LTP)*

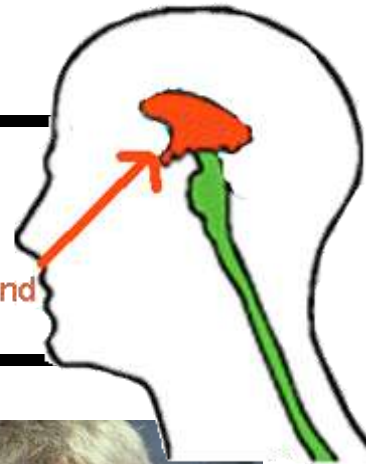


event

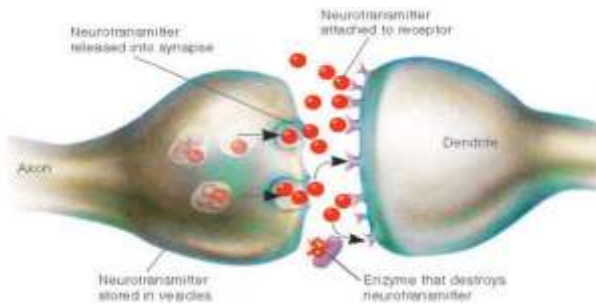
'le temps perdu'

emotion

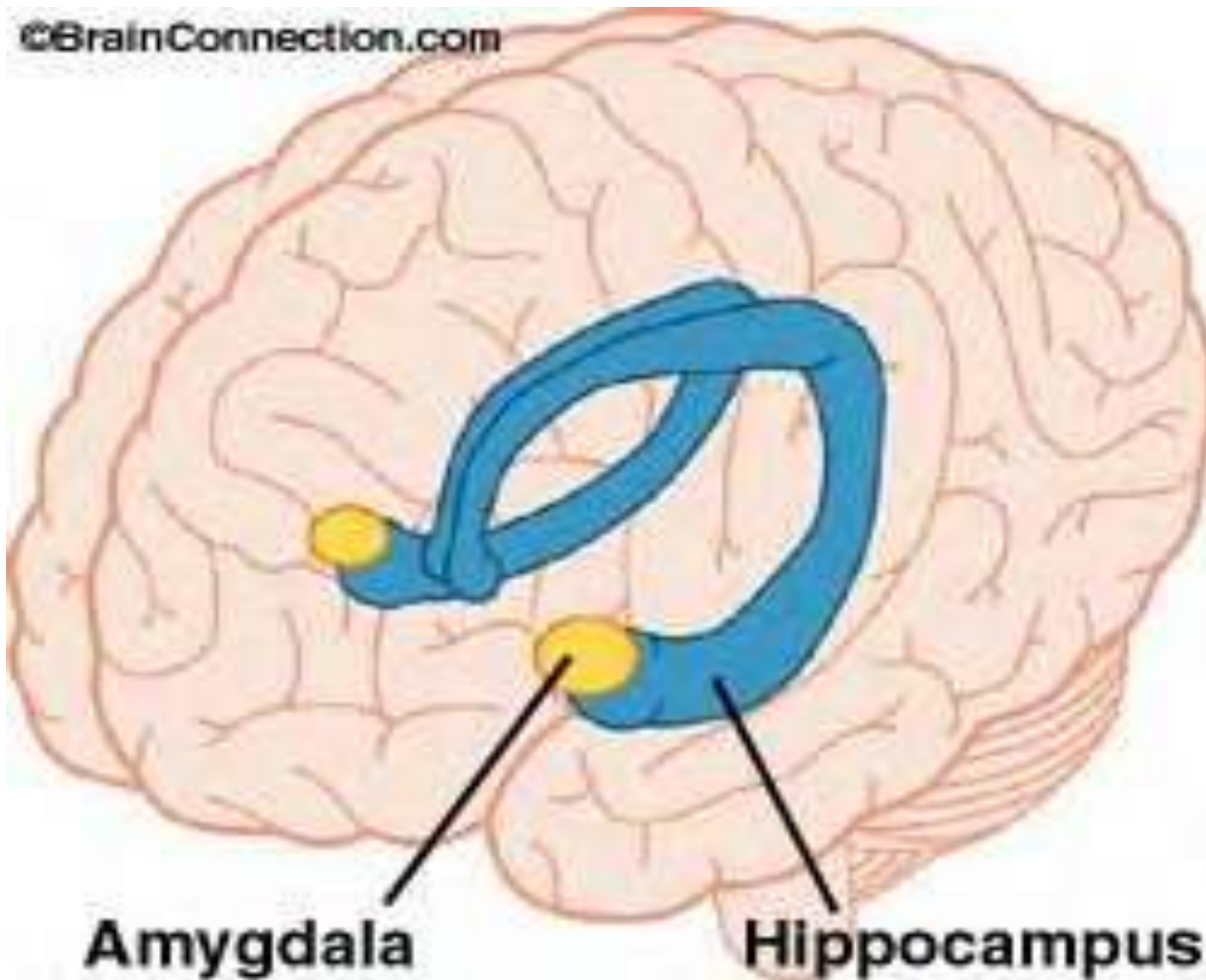
Limbic Brain
Emotional Mind



memory



epigenetics



emotional 'hard disc'
memory

mind's eye imagination
virtual reality



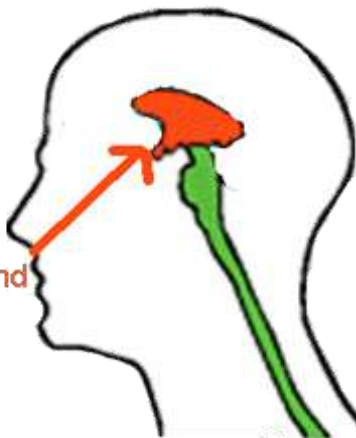
fear conditioning



emotion hijacks the cortex
neurological primacy

'In two minds'

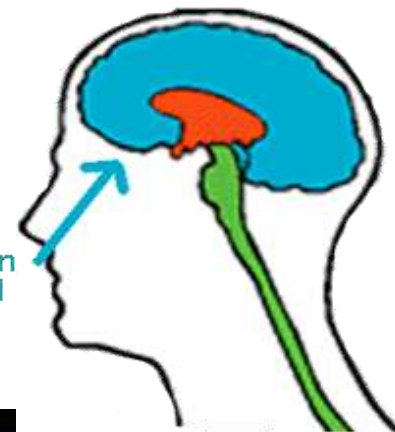
Limbic Brain
Emotional Mind



Primitive Emotional Brain
drives cognition, physiology
& behaviour

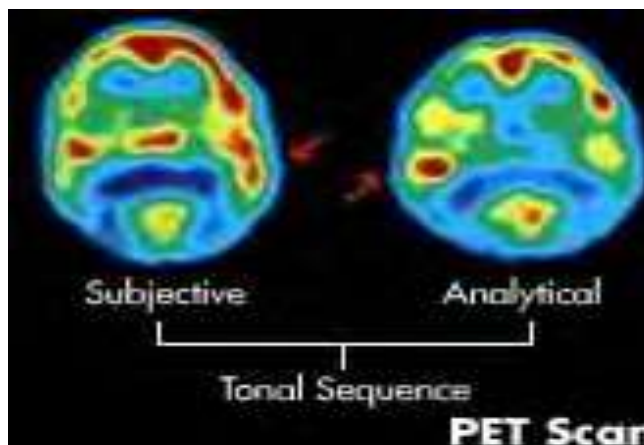


Neocortex Brain
Analytical Mind



Thinking Brain

Rational
Logical
Analytical
Objective



18.11.2010

www.positivecvt.co.uk

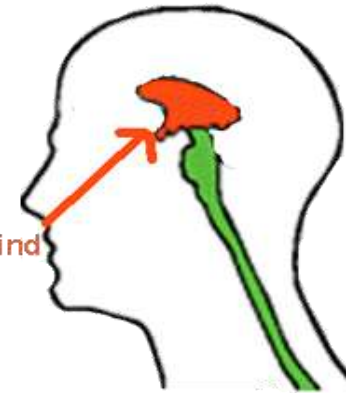
positive+

Triggers

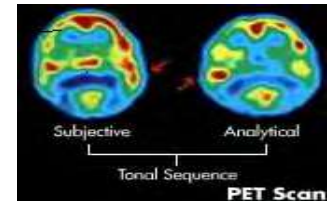


internal & external
events - triggers

Limbic Brain
Emotional Mind

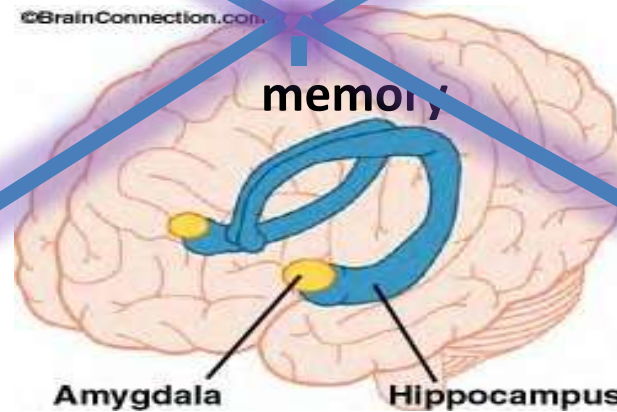


emotion



©BrainConnection.com

memory



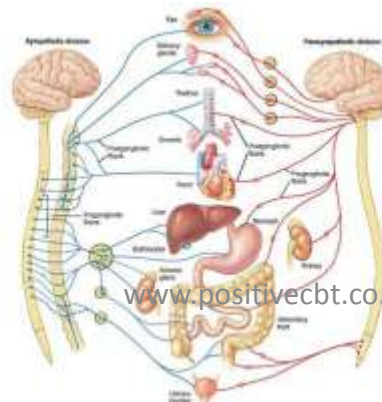
behaviour

thoughts
&
images

rumination



physiology



www.positivecvt.co.uk



positive+

Negative affective states

Meta emotion

Meta cognition



Rumination – ‘attentional bias’ - focus on threat

‘chronic embitterment syndrome’



CNS Sensitisation



Sensitisation is a basic form of learning and memory

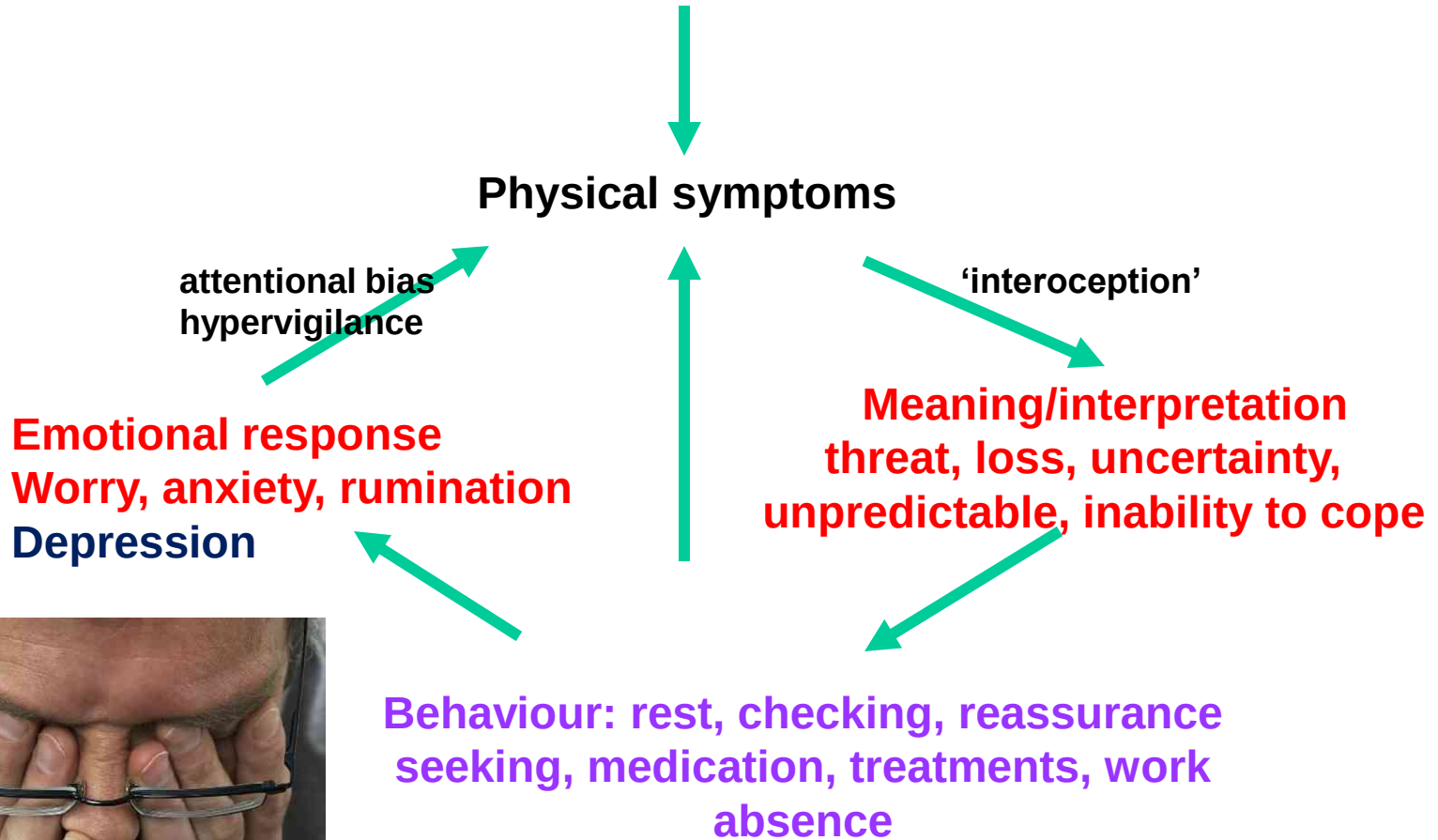
Fear can be conditioned with one severe noxious exposure or by repeated mild exposure – ‘**kindling**’ (especially in the absence of control).

- Exposure to noxious stimuli without control increases anxiety and drops mood – ‘**depressogenic**’
- Anxiety and depression lower the pain threshold – **hyperalgesia**.

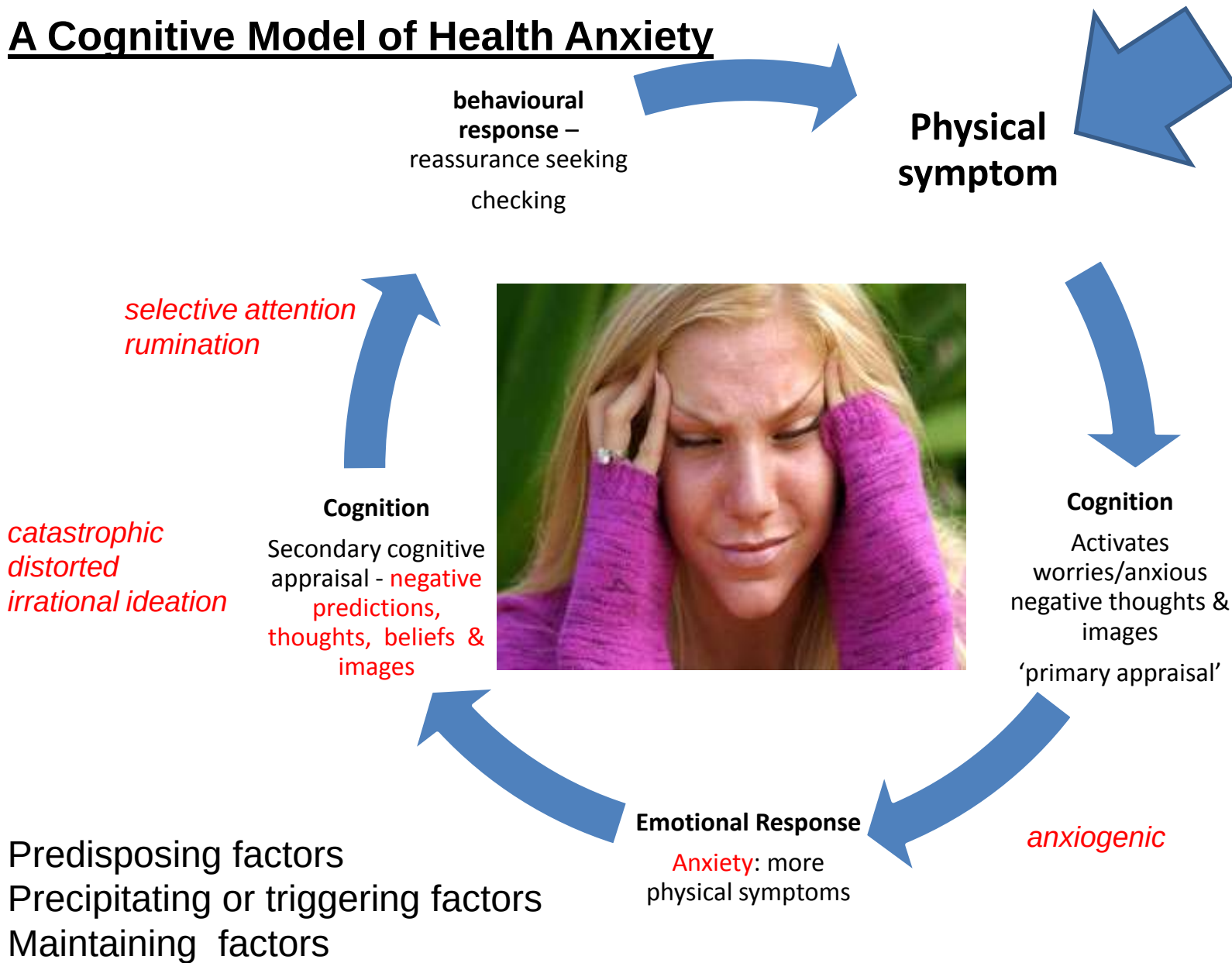
cognitive model of health anxiety

Triggers

normal variations, stress, distress, anxiety, illness,



A Cognitive Model of Health Anxiety



47 year old man
return trip from US
stress, fatigue,
caffeine, alcohol

Symptoms

palpitations
hot sweaty
breathless



Cognition

*'Why am I feeling
like this'?*

*'This could be
serious'*

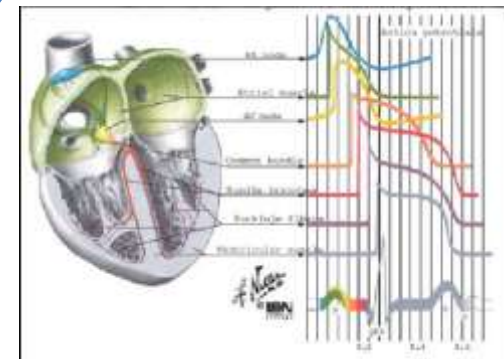
*'I have a family
history of heart
disease'*

worry/anxiety
adrenaline/hyperventilation
symptom focusing

Increased Symptoms

palpitations,
dizziness, chest
tightness, sweaty,
more breathless.

distorted
irrational
catastrophic ideation



Cognition

*'I am having a
heart attack'*

*'I am going to
die'*

Anxiety & Adrenaline

Increased symptoms

breathlessness,
chest tightness
palpitations
dizziness
'spaced'
'detached'

Behaviour

Panic
Call for help
Sedated on plane

avoidance

reassurance seeking

Consequences

11 hospital admissions

17 consultations in primary care

29 consultations in secondary care – cardiological & neurological

CT; MRI; Echocardiograms; Treadmill; angiogram;

£27,000 visit to the Mayo Clinic

Symptoms triggered by stress; frustration/anger; heat; exercise and activity

Stopped work – income protection.

Evidence based Rx CBT for health anxiety & situational avoidance

graded return to work over 3 months



34 year old woman
recent third miscarriage
pressure at work
criticised by boss
perfectionistic
poor sleep

stress
worry
anxiety

Symptoms
dizziness,
numbness –
hands and face,
pins and needles

Cognition

'This is really strange'

'Why do I feel like this'

*'There is something serious the matter
with me'*

worry/anxiety
adrenaline/hyperventilation
symptom focusing

Symptoms
'feeling strange,
spaced,
detached, weird,
& worsening
dizziness

anxiety & adrenaline

Cognition

*'I am having a
stroke'*

*'I have got a brain
tumour'*

*'this could be
MS'*

Behaviour
GP, neurologist,
investigations ++
NAD



Dizziness, numbness, pins and needles – anxiety re MS

Neurologist: MRI scan; VER's; LP. All normal Bloods = nad

'nothing the matter' 'possible virus'

lack of credible explanation creating 'uncertainty'

'unpredictability' – symptoms recur – creating escalating worry/anxiety

poor sleep, worsening physical symptoms – 'sensitised' to symptoms

stopped work – income protection

Stress, anxiety, chronic fatigue – diagnosed CFS/ME by GP.

web search re CFS/ME – worsening anxiety & concerns about long term health

clinically depressed: felt disbelieved by doctors/employer/some friends & family



Evidence based Rx

CBT; graded exercise, graded return to work



**The mind is its own place, and in itself
Can make a Heav'n of Hell, or Hell of Heav'n.**

John Milton, Paradise Lost (1667)

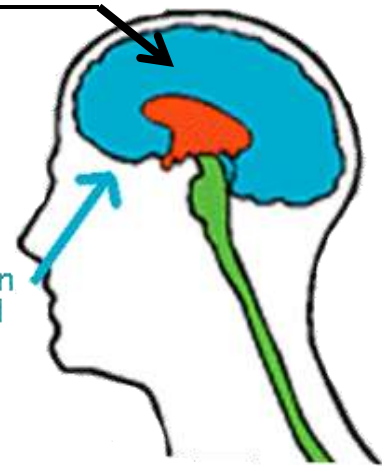
The greatest discovery of my generation is that man can alter his life simply by altering his attitude of mind.

*William James
philosopher & psychologist
(1842 - 1910)*



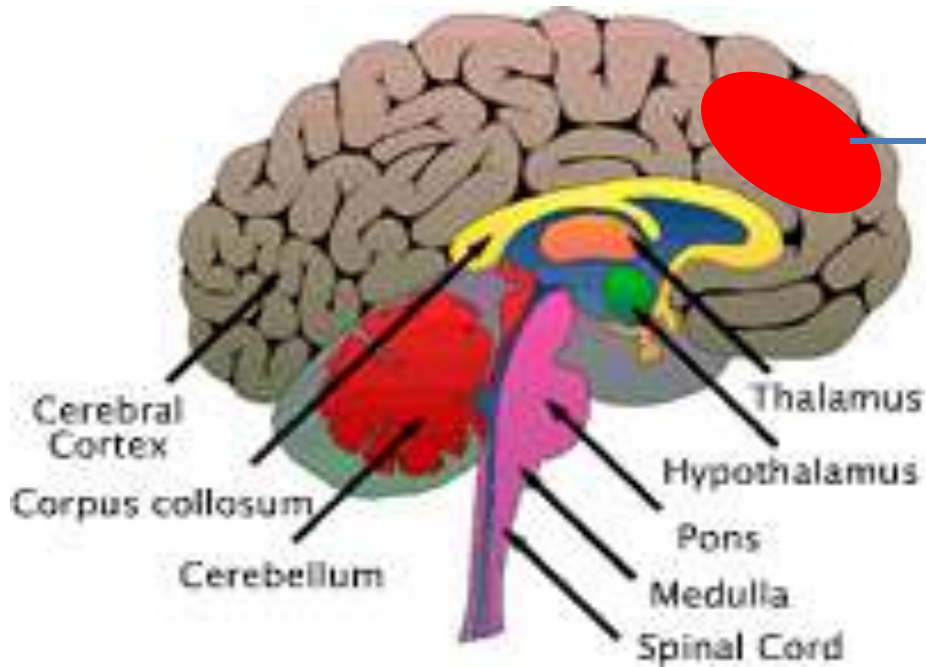
inhibitory pathways
dampen emotional
responses

Neocortex Brain
Analytical Mind



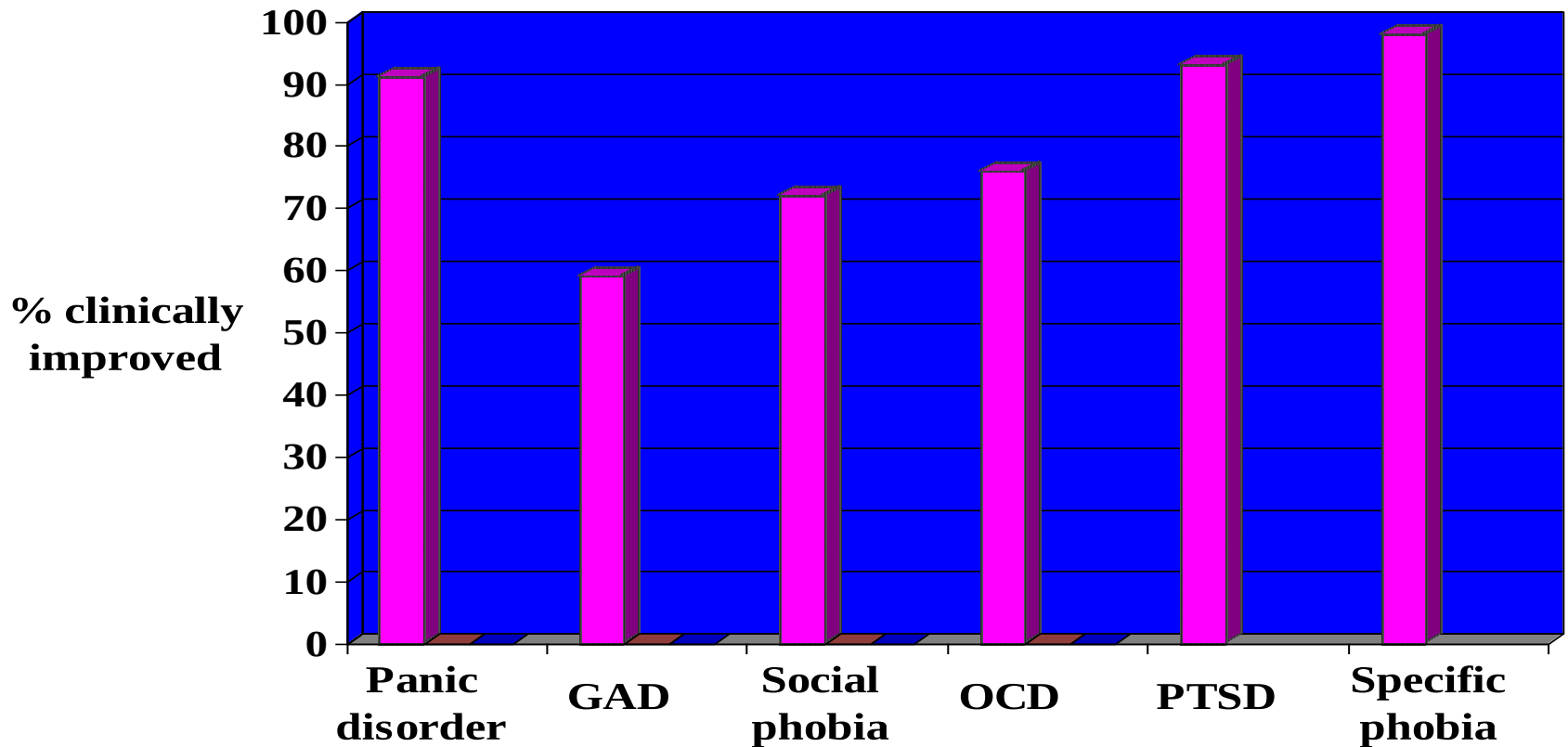
cognitive reframing; behavioural experiments
extinguish the conditioned response

pre-frontal
cortex



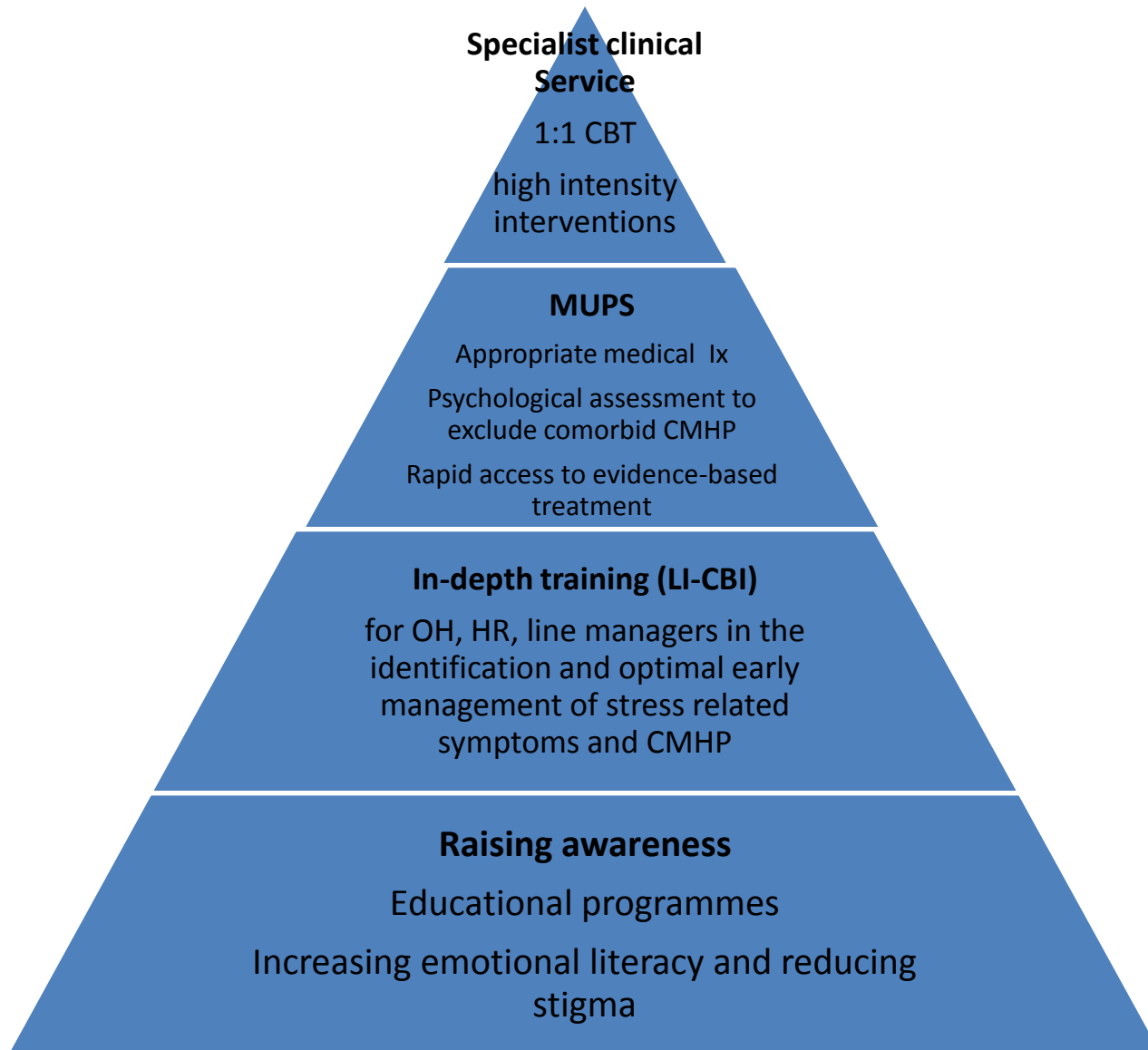
Cognitive Behavioural Therapy (CBT) is an evidence-based treatment: CFS/ME; IBS; chronic pain; depression, anxiety

Early Access to effective treatment



Stepped Care Pathway:

Prevention and early intervention





Dr Brian Marien

Positive Health Strategies Ltd.

Temple Chambers
First Floor Rooms 125-130
3-7 Temple Avenue
London EC4Y 0HP

020 7936 3454

www.positivehealthstrategies.com

T